

RESEARCH BRIEF

Topic: Investigating Medicaid Claim Denials

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Purpose: An introductory reference guide explaining Medicaid claim denial drivers, denial impacts, and resolution workflows for onboarding staff and maintaining consistent billing operations.

Executive Summary

Timely Medicaid reimbursement depends on accurate documentation, compliance with payer requirements, and complete supporting records. When claims are denied, identifying the underlying cause often requires gathering information from multiple systems and documents, reconciling discrepancies, and preparing evidence-based summaries for review by managed care organizations (MCOs).

Author's Role: As Revenue Cycle Manager and an Analyst, I routinely investigate Medicaid claim denials by analyzing payer responses, authorization records, client documentation, and billing history to determine root causes, prepare supporting documentation for claim reconsiderations and recommend corrective actions.

Information Sources Analyzed

Source	Purpose
Medicaid Remittance Advice (EOB)	Identify specific denial and remark codes
MCO Portal	Validate authorizations and explanation of benefits
Client Records	Verify authorization details and service delivery dates
Case Manager Documentation	Confirm supporting documentation
Internal Billing Database	Identify historical claim patterns and recurring trends
Medicaid Eligibility Portals	Verify beneficiary coverage during specific dates of service
MCO Correspondence	Document official payer communications and formal resolutions

Research Objective

Develop a standardized investigation framework that enables staff to:

- Consistently identify root causes of claim denials
 - Gather and validate supporting evidence across multiple systems
 - Prepare clear, evidence-based documentation for reconsiderations and appeals.
 - Track and categorize recurring denial trends over time.
 - Recommend operational process improvements to reduce future denial rates.
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Observed Denial Patterns

Analysis of recurring claim denials identified distinct patterns among the three Managed Care Organizations (MCOs) supported by the Revenue Cycle team. Denials were grouped according to their verified root cause rather than their initial surface-level denial code:

Category	Observed Frequency	Primary Cause	Responsibility
Prior Authorization	85%	Services rendered prior to authorization generation or unlinked 837P files	Shared (Provider / MCO)
Eligibility & Coverage	10%	Missed Medicaid renewals, or CHIP funding conflicts.	Client / Guardian
System Processing	3%	MCO-side processing glitches or clearinghouse submission errors.	Payer
Coding Errors	2%	Discrepancies in ICD-10 diagnosis or CPT procedure codes.	Provider

Operational Impact Breakdown:

- **Prior Authorization (85%):** Driven primarily by timing lags between service delivery and MCO code generation. Additionally, recurring MCO-side authorization processing glitches caused repeated denials despite valid authorizations being on file.

- **Eligibility Issues (10%):** Caused by coverage lapses during transition from private pay to Medicaid, or acceptance of out-of-county clients under CHIP (Children's Health Insurance Program) funding.
- **System Processing & Coding (5% combined):** System crashes during batch submissions and occasional manual entry errors on billing codes.

Research Process

Denial trends were identified through routine analysis of Medicaid remittance advice, MCO correspondence, authorization records, billing history, and internal client documentation. Denials were categorized by root cause after verifying information across multiple independent sources. Percentages represent observed patterns over recurring billing cycles rather than a formal statistical sample.

Recurring Authorization System Failures

Over multiple billing cycles, recurring authorization-related denials were observed despite verification that valid authorizations existed within the MCO's system. These denials differed from routine errors because they affected large batches of claims simultaneously across multiple providers, indicating a system-level glitch at the MCO rather than isolated billing errors.

- **Operational Impact:** During these events, 20% to 45% of submitted claims required manual investigation, delaying reimbursement.
 - **Resolution Workflow:** Claims were researched individually, supporting proof was compiled, and structured findings were escalated to the MCO. Batch reconsideration requests were submitted with ongoing tracking until the MCO confirmed system corrections.
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Recommendations & Key Takeaways

Immediate Actions

- Cross-reference provider authorizations prior to claim submission.

- Log all reconsideration claim IDs in a centralized database for audit tracking.
- Verify receipt and storage of required client choice forms.

Long-Term Improvements

- Standardize authorization verification procedures across all MCO contracts.
- Track and log recurring denial patterns by specific payer.
- Develop internal documentation checklists for onboarding staff.

Core Research Takeaway:

Effective denial resolution depends on validating information across multiple independent sources rather than relying solely on surface-level denial codes. Root-cause analysis transforms isolated billing errors into actionable insights that drive long-term process improvements.